Termination of Resuscitation

AC-12 | Adult Cardiac

Quick flow

01 Adult Nontraumatic OHCA [EMT / AEMT / Paramedic]

Begin AC-03/AC-11 care • confirm no special-circumstance exclusion

02 Complete Resuscitation [EMT / AEMT / Paramedic]

At least 20 min high-quality care • rhythm pathway • airway/ventilation • access/medications • reversible causes

03 Universal TOR Criteria [EMT / AEMT / Paramedic]

Not witnessed by EMS • no shock before transport • no ROSC before transport

04 All 3 Present? [EMT / AEMT / Paramedic]

No: continue resuscitation and consider transport • Yes: evaluate full clinical context

05 Medical Control Required [EMT / AEMT / Paramedic / Medical Control]

Report criteria, exclusions, resuscitation, rhythms, $EtCO_2$ trend, and reversible causes

06 Termination Authorized? [EMT / AEMT / Paramedic]

Yes: stop CPR, record time, support family, preserve scene • No/unavailable: continue and transport

07 Document + Disposition [EMT / AEMT / Paramedic]

Times • criteria • interventions • physician order • notifications • family support • scene disposition

Overview

- Field termination pathway for an adult, nontraumatic, out-of-hospital cardiac arrest after a complete resuscitation and application of the Universal Termination of Resuscitation rule.
- Termination requires Medical Control authorization. When any required criterion is absent or an exclusion is present, continue resuscitation and consider transport.

Provider-Level Actions

All levels continue resuscitation while criteria are evaluated; no provider stops CPR solely from the checklist.

EMT

- Continue high-quality CPR, AED use, ventilation, timing, and documentation while determining EMS witness status, shocks delivered, and any ROSC.
- Identify special circumstances and immediately report any exclusion or possible reversible cause.
- Do not terminate resuscitation without a documented Medical Control order unless a separate obvious-death or valid DNR policy applies.

AEMT

- Perform all EMT actions plus indicated vascular access, arrest medication, airway support, and reversible-cause treatment.
- Confirm the duration and adequacy of resuscitation and assist with the Medical Control report and order read-back.
- Continue resuscitation and transport when any Universal TOR criterion is absent, an exclusion is present, or Medical Control is unavailable.

Paramedic

- Confirm that at least 20 minutes of documented resuscitation, rhythm-specific care, and reversible-cause evaluation have been completed.

- Confirm all three Universal TOR criteria and absence of exclusions before requesting termination authorization.
- Provide the complete resuscitation summary to Medical Control, repeat the order back, direct termination when authorized, and coordinate disposition.

Medical Control

- Review eligibility, Universal TOR criteria, exclusions, resuscitation course, rhythms, shocks, ROSC, $EtCO_2$ trend, reversible causes, and operational circumstances before authorizing termination.

Indications

- Adult, nontraumatic, out-of-hospital cardiac arrest with persistent pulselessness after at least 20 minutes of documented high-quality resuscitation.
- All three Universal TOR criteria are present and Medical Control authorization is available.

Contraindications

- EMS-witnessed arrest, any shock delivered before transport, or any ROSC before transport.
- Pediatric cardiac arrest, traumatic arrest, overdose or suspected toxicologic arrest, pregnancy, hypothermia, drowning/submersion, lightning/electrocution, or in-hospital cardiac arrest.
- A reversible special circumstance that warrants continued treatment or transport.
- Medical Control declines termination or cannot be reached.

Assessment | Universal Termination of Resuscitation rule

- The arrest was not witnessed by EMS personnel.
- No shock was delivered before transport.
- No return of spontaneous circulation occurred before transport.
- All three criteria must be present. If any criterion is absent, continue resuscitation and consider transport.

Assessment | Required resuscitation before consideration

- Complete at least 20 minutes of documented high-quality CPR and rhythm-specific resuscitation before applying the Claiborne termination pathway.
- Provide appropriate defibrillation, ventilation, airway management, vascular access, arrest medications, and reversible-cause treatment without interrupting compressions.
- Review witness information, bystander care, downtime, all rhythms, shocks, transient pulse or movement, medication response, airway position, $EtCO_2$ trend, and possible reversible causes.
- The 20-minute safeguard does not independently establish futility and does not replace the Universal TOR criteria or Medical Control decision.

Assessment | Exclusion screen

- Do not use this pathway for pediatric, traumatic, overdose/toxicologic, pregnancy-associated, hypothermic, drowning/submersion, lightning/electrocution, or in-hospital arrest.
- Use separate obvious-death, valid DNR/advance-directive, traumatic-arrest, and special-circumstance protocols when applicable.
- When history or circumstances are uncertain, continue resuscitation, consult Medical Control, and transport as directed.

Treatment steps

- Continue AC-03 and AC-11 resuscitation while termination eligibility is evaluated.
- After at least 20 minutes of documented high-quality resuscitation, confirm that the arrest was not witnessed by EMS, no shock was delivered before transport, and no ROSC occurred before transport.

- If any Universal TOR criterion is absent, continue resuscitation and consider transport.
- Confirm that no exclusion or reversible special circumstance requires continued treatment or transport.
- Contact Medical Control and report the patient, circumstances, Universal TOR criteria, exclusions, duration, rhythms, shocks, medications, airway, $EtCO_2$ trend, reversible causes, and response to treatment.
- If termination is authorized, repeat the order back, stop resuscitation, confirm persistent apnea and pulselessness, and document the termination time.
- If termination is declined or Medical Control is unavailable, continue resuscitation and transport unless a separate obvious-death or valid DNR policy applies.
- Support family and witnesses, explain the process compassionately, preserve the scene when investigation is possible, and coordinate disposition and required notifications.

Warnings / precautions

- $EtCO_2$ must never be the sole reason to terminate resuscitation.
- In an intubated adult, $EtCO_2$ below 10 mmHg after 20 minutes may contribute only as one component of a multimodal decision.
- Do not apply a specific $EtCO_2$ cutoff to a nonintubated patient or a patient ventilated through a supraglottic airway.
- Do not terminate when any Universal TOR criterion is absent, an exclusion is present, or Medical Control authorization has not been obtained.
- Do not remove tubes, vascular access, clothing, or other potential evidence when the death may require medical-examiner or law-enforcement investigation.

Clinical pearls

- The Universal TOR rule is intended for a mixed BLS/ALS or tiered EMS response system.
- A delivered shock, transient ROSC, or EMS-witnessed arrest removes the patient from this field-termination pathway.
- Low $EtCO_2$ should first prompt reassessment of compression quality, ventilation rate, airway placement, and reversible causes.
- Use clear team communication and compassionate family support before and after the termination decision.

Special populations | Separate pathways required

- Use TB-10 for traumatic arrest, TE-07 for suspected overdose/toxicologic arrest, TE-05 for hypothermia, TE-03 for drowning/submersion, and AO-03 for pregnancy-associated emergencies.
- Consult Medical Control early for any uncertain cause or exceptional circumstance.

Special populations | Obvious death or valid DNR

- Obvious-death criteria and valid DNR/advance-directive decisions are governed by separate policy and do not require completion of the Universal TOR pathway.

Related pathways / contacts

AC-03 Adult Cardiac Arrest - Continue the primary adult cardiac-arrest pathway while eligibility is evaluated.

AC-09 VF / Pulseless VT - Open for any shockable rhythm; a delivered shock excludes Universal TOR.

AC-11 TEAM-Focused CPR - Open for role assignment, CPR quality, timing, and documentation.

TB-10 Traumatic Arrest - Use instead for arrest caused by trauma.

TE-03 Drowning / Submersion - Use instead for drowning or submersion-related arrest.

TE-05 Hypothermia - Use instead for suspected hypothermic arrest.

TE-07 Overdose / Toxic Ingestion - Use instead for suspected overdose or toxicologic arrest.

AO-03 OB-GYN Emergency - Open for pregnancy-associated arrest or another obstetric emergency.

Source and references

American Heart Association. 2025 Guidelines for CPR and ECC: Adult Advanced Life Support - Termination of Resuscitative Measures. | American Heart Association. 2025 BLS/Universal Termination of Resuscitation Rules. | American Heart Association. 2025 ALS Termination of Resuscitation Rule. | American Heart Association. 2025 Guidelines for CPR and ECC: Ethics. | Claiborne EMS adult cardiac-arrest, special-circumstance, death-in-the-field, and disposition policies.

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